BASELINE → TREATMENT → FOLLOW-UP

Checks across treatment phases

Relative stages, not equally spaced dates. No patient dates are needed. Pancreatic enzyme checks supplement National Cancer Control Programme (NCCP) from the current product information; the team confirms timing and reviews every result.

Check	Before starting	Induction Day 1	Induction Days 2–3	Maintenance	After treatment
Blood counts, organs and hormones	full blood count (FBC), kidney/liver, glucose, thyroid and pancreatic enzymes; baseline suitability	Review symptoms and cycle bloods before dosing	Report symptoms between visits; extra tests if needed	Cycle bloods; thyroid every 3–6 weeks; pancreatic enzymes periodically/as needed	Follow unresolved changes and the team's plan
Kidneys, hearing and other checks	Measured GFR preferred; audiology if indicated	Validate renal dose and recent creatinine	Report low urine, hearing changes or weakness	electrocardiogram (ECG), cortisol, lipids and audiology as indicated	Persistent or new problems still need review
Support and route	Confirm intravenous (IV) or oral etoposide route; written anti-sickness plan	Carboplatin-specific high-risk anti-sickness plan	Etoposide on both days; reconcile support through two days afterwards	No routine induction chemotherapy antiemetics	Keep the current medicine and contact plan
Immune effects and cancer review	Record symptoms; explain immunotherapy alert card	Report infusion symptoms immediately	Immune symptoms can start any day	Continue symptom vigilance; scans per consultant plan	Delayed immune effects remain possible

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00908 v1b · official regimen · EMA Hetronifly SmPC · updated8October2026

THE WHOLE TREATMENT JOURNEY

Serplulimab, carboplatin and etoposide: treatment and checks

First-line extensive-stage small-cell lung cancer

Induction lasts four planned 21-day cycles unless treatment needs to stop earlier. On Day 1 you receive serplulimab, carboplatin and etoposide by drip. With the standard IV schedule, etoposide is repeated on Days 2 and 3. After induction, carboplatin and etoposide stop. Serplulimab alone can continue every three weeks while it is helping and its risks remain acceptable. There is no fixed universal number of maintenance cycles in this NCCP regimen. Your team may instead prescribe etoposide by drip on Day 1 and capsules twice daily on Days 2 and 3. This is a separate alternative, not extra treatment. Oral and IV doses are not interchangeable milligram for milligram. Confirm which plan you have; take the capsules on an empty stomach as prescribed.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Induction Day 1	Induction Days 2–3	Maintenance	After treatment
Treatment NCCP 00908	Confirm suitability and chosen etoposide route	Serplulimab → carboplatin → etoposide	Etoposide IV each day or prescribed oral BID alternative	Serplulimab alone every21days after four induction cycles	Follow-up or next treatment per team
Blood counts, organs and hormones NCCP 00908 · SERPLULIMAB-SMPC	FBC, kidney/liver, glucose, thyroid and pancreatic enzymes; baseline suitability	Review symptoms and cycle bloods before dosing	Report symptoms between visits; extra tests if needed	Cycle bloods; thyroid every 3–6 weeks; pancreatic enzymes periodically/as needed	Follow unresolved changes and the team's plan
Kidneys, hearing and other checks NCCP 00908 · SERPLULIMAB-SMPC	Measured GFR preferred; audiology if indicated	Validate renal dose and recent creatinine	Report low urine, hearing changes or weakness	ECG, cortisol, lipids and audiology as indicated	Persistent or new problems still need review
Support and route NCCP 00908 · SERPLULIMAB-SMPC	Confirm IV or oral etoposide route; written anti-sickness plan	Carboplatin-specific high-risk anti-sickness plan	Etoposide on both days; reconcile support through two days afterwards	No routine induction chemotherapy antiemetics	Keep the current medicine and contact plan
Immune effects and cancer review NCCP 00908 · SERPLULIMAB-SMPC	Record symptoms; explain immunotherapy alert card	Report infusion symptoms immediately	Immune symptoms can start any day	Continue symptom vigilance; scans per consultant plan	Delayed immune effects remain possible

What each check is for

1 Baseline and route

Before the first cycle

Confirm suitability and correct schedule

Ask / **act:** Document IV versus oral etoposide, GFR method, medicines and urgent contacts.

Sources: NCCP 00908

2 Induction to maintenance

After four planned induction cycles

Chemotherapy and routine support stop

Ask / **act:** Confirm maintenance plan and continue cycle bloods and immune checks.

Sources: NCCP 00908

3 Treatment pause or completion

For toxicity, progression or agreed change

Delayed effects and ongoing care

Ask / **act:** Give a follow-up, hormone-replacement if needed and urgent-contact plan.

Sources: NCCP 00908

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-26 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.23